

16. ACOG 206: Hormonal Contraception With Coexisting Medical Conditions

Companion reading handout for simultaneous listening.

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Source: ACOG Practice Bulletin No. 206, *Use of Hormonal Contraception in Women With Coexisting Medical Conditions*.

The dangerous way to learn contraception in medically complex patients is to memorize one disease at a time. Hypertension, migraine, lupus, diabetes, postpartum, breast cancer, antiepileptic drugs. That kind of list collapses under real clinic conditions, because the same disease name can hide several different risks, and the same contraceptive can be useful in one mechanism and unsafe in another.

The more reliable approach is to identify the lever being pulled.

One lever is estrogen risk. Estrogen can increase hepatic production of coagulation-related proteins, including factor seven, factor ten, and fibrinogen. That matters in thrombophilia, prior clot, early postpartum physiology, major surgery with immobilization, antiphospholipid-positive lupus, migraine with aura, severe hypertension, and vascular disease.

Another lever is progestin-specific risk. Progestin-only methods avoid estrogen, but depot medroxyprogesterone acetate, progestin-only pills, the etonogestrel implant, and the levonorgestrel IUD have different pharmacology and different clinical weak points.

A third lever is efficacy. Some drugs and surgeries make contraception fail without making it toxic. Rifampin, rifabutin, enzyme-inducing antiepileptics, some antiretrovirals, and malabsorptive bariatric surgery belong here.

A fourth lever is pregnancy risk. A contraceptive method should never be judged against imaginary zero risk. The comparison is the method's risk against the risk of unintended pregnancy in that specific patient. ACOG emphasizes this because about eighty-five percent of sexually active women become pregnant within one year if they use no contraception.

Those four levers, estrogen risk, progestin-specific risk, efficacy risk, and pregnancy risk, organize the whole bulletin.

The language used for that organization is the U S Medical Eligibility Criteria, or USMEC.

Category one means no restriction for use of the method.

Category two means the advantages generally outweigh theoretical or proven risks. The patient can usually use the method, with the caveat understood.

Category three means the theoretical or proven risks usually outweigh the advantages. Category three still leaves room for individualized care when safer alternatives are unavailable, unacceptable, or less effective for the patient. It demands counseling and follow-up.

Category four means unacceptable health risk. The method should be avoided.

Two details make USMEC clinically useful. First, the category is attached to a pair: a patient condition and a contraceptive method. Migraine with aura is category four for estrogen-containing contraception, while progestin-only methods are category one. Second, when several conditions coexist, the highest category usually drives the decision. A twenty-five-year-old with migraine with aura and rheumatoid arthritis does not average the risks. The aura controls the combined hormonal contraception decision.

Now build the method map.

Combined hormonal contraception, or CHC, means estrogen plus progestin. It includes the combined pill, the transdermal patch, and the vaginal ring. The ring is not treated as a local method for estrogen risk. The patch is not treated as a small exposure loophole. ACOG states that contraindications to combined oral contraceptives for venous thromboembolism risk should be considered to apply to the patch and the ring.

Progestin-only methods include depot medroxyprogesterone acetate, or DMPA, the etonogestrel implant, progestin-only pills, and levonorgestrel-releasing IUDs. These methods share the absence of estrogen. They do not share the same efficacy behavior, bleeding profile, lipid concerns, or drug-interaction vulnerability.

Nonhormonal high-efficacy methods include the copper IUD and sterilization. In medically complex patients, the copper IUD is often a central option because it preserves high contraceptive efficacy while avoiding both estrogen and progestin exposure.

The first major clinical zone is clot risk.

The estrogen component of combined hormonal contraception increases hepatic production of coagulation-related serum globulins. ACOG specifically names factor seven, factor ten, and fibrinogen. All combined hormonal methods increase venous thromboembolism risk. The important proportional fact is that this risk remains about half as high as the elevated VTE risk observed in pregnancy. That is why a clinician should select a safer method rather than leave the patient unprotected from pregnancy.

The VTE risk conditions that push combined hormonal contraception toward category three or four include smoking at age thirty-five or older, the early postpartum period, major surgery with prolonged immobilization, history of deep vein thrombosis or pulmonary embolism, hereditary thrombophilia including antiphospholipid syndrome, active or complicated inflammatory bowel disease, systemic lupus erythematosus with positive or unknown antiphospholipid antibodies, and superficial venous thrombosis.

Major surgery teaches the timing physiology well.

Combined hormonal contraception is category four for major surgery with anticipated prolonged immobilization.

If the patient is expected to be ambulatory after surgery, ACOG says there is no reason to stop combined hormonal contraception, category two.

If stopping is chosen for a high-risk operation, the clotting-factor normalization associated with discontinuation is seen only when combined contraception is stopped four to six weeks before surgery.

Stopping it two days before the operation gives a false sense of risk reduction.

A prior VTE requires subtyping. A remote single clot provoked by a nonrecurring factor, such as immobilization after a motor vehicle accident years earlier, may leave combined hormonal contraception in category three if no other method is available or desired. Known familial thrombophilia is different. Known thrombogenic mutations make combined hormonal contraception category four. Progestin-only methods and levonorgestrel IUDs are category two.

Routine thrombophilia screening before combined contraception is not recommended.

Screening would identify about five percent of U S oral contraceptive candidates as factor V Leiden carriers, yet most would never experience a VTE even if they used combined oral contraception.

One analysis concluded that screening more than one million combined oral contraceptive candidates would prevent, at best, two oral contraceptive-associated deaths.

That number is memorable because it explains why a biologically real risk does not automatically justify universal screening.

Factor V Leiden also shows how risk multiplies. Heterozygous factor V Leiden raises baseline VTE risk about sevenfold. Heterozygous carriers using oral contraceptives have risk more than fifteen to thirty times higher than noncarrier women using nonhormonal contraception. The clinical conclusion remains targeted: known thrombophilia makes estrogen category four; routine population screening remains unsupported.

Progestin-only options are central in clot-risk patients.

Progestin-only pills, the implant, and the levonorgestrel IUD are appropriate to initiate in women with a history of or risk of VTE, myocardial infarction, or stroke, category two.

DMPA needs a separate mental slot.

USMEC allows DMPA in women at VTE risk, category two, but DMPA becomes category three in personal ischemic heart disease or stroke because of hypoestrogenic effects and increased total cholesterol.

DMPA avoids estrogen, but it is metabolically active.

Lupus is the second zone, and lupus should be taught as an antibody-status decision.

Women with SLE have increased arterial and venous thrombosis compared with the general population. Thrombosis is a major cause of death in SLE, and persistently positive antiphospholipid antibodies increase that risk further. ACOG also notes that young women with SLE have an estimated fifty-fold increased risk of myocardial infarction compared with age-matched and sex-matched controls.

Before hormonal contraception is initiated in a woman with SLE, antiphospholipid antibody status should be checked. Positive antiphospholipid antibodies make combined hormonal contraception category four. Stable or inactive SLE without antiphospholipid antibodies and without other cardiovascular risk factors can allow combined hormonal contraception, category two.

Progestin-only contraception in antiphospholipid-positive lupus is a useful caution against sloppy teaching. Progestin-only methods and levonorgestrel IUDs are category three in SLE with antiphospholipid antibodies. That category does not come from the same estrogen hepatic coagulation mechanism. It comes from the patient's high baseline thrombosis propensity.

Severe thrombocytopenia adds a bleeding mechanism. Platelets below fifty thousand per microliter make initiation of DMPA category three, because DMPA initiation can produce irregular bleeding. Immunosuppression adds a separate IUD concern, and ACOG states that levonorgestrel IUDs are safe in immunosuppressed women with SLE, category two.

Now move to the postpartum clock.

Postpartum counseling has three questions: how many days after birth, breastfeeding or not, and whether additional VTE risk factors are present.

Most nonbreastfeeding women will not ovulate until six weeks postpartum, but ovulation can occur as early as three weeks. Exclusive breastfeeding without formula supplementation can reduce pregnancy risk for at most six months through the lactational amenorrhea method. Those facts justify early contraception counseling. They do not justify assuming safety from pregnancy.

Combined hormonal contraception is category four during the first twenty-one days after birth, regardless of breastfeeding status.

The reason is the early postpartum clot-risk peak.

From day twenty-one through day forty-two, risk decreases, but additional VTE risk factors keep combined hormonal contraception category three.

These risk factors include age thirty-five or older, previous VTE, thrombophilia, immobility, transfusion at delivery, peripartum cardiomyopathy, body mass index thirty or higher, postpartum hemorrhage, cesarean delivery, preeclampsia, and smoking.

Breastfeeding adds the lactation layer.

Breastfeeding women should avoid combined hormonal contraception through the first thirty days: category four through day twenty-one, then category three through day thirty.

From day thirty to forty-two, combined hormonal contraception is category two only if additional VTE risk factors are absent, and category three if they are present.

After forty-two days, breastfeeding women are category two for combined hormonal contraception because the benefits of contraception likely outweigh theoretical effects on milk supply.

Nonbreastfeeding women have no postpartum restriction on combined hormonal contraception after forty-two days, category one.

The postpartum VTE number should be held carefully. VTE risk does not return to baseline until twelve weeks postpartum. The absolute risk after six weeks is low: twenty-two point one cases per one hundred thousand deliveries within the first six weeks, compared with three point zero cases per one hundred thousand deliveries during weeks seven through twelve. The table thresholds therefore mark eligibility decisions, while the biology fades gradually.

Progestin-only methods behave differently postpartum. Nonbreastfeeding women can use progestin-only methods immediately after delivery, category one. Breastfeeding women can use progestin-only methods at any time; the first thirty days are category two, and after thirty days category one.

The reason for the first-thirty-day category two in breastfeeding is lactogenesis.

After placental delivery, progesterone withdrawal is thought to help trigger prolactin secretion and lactogenesis.

Pharmacologic progestin immediately after birth raises a theoretical concern that it could preempt that signal.

Once lactogenesis has begun, generally in the first forty-eight to seventy-two hours postpartum, there is general agreement that progestin use does not affect

breastfeeding performance.

Observational studies are reassuring even with immediate initiation, showing no effect on successful breastfeeding initiation, continuation, or infant growth and development, while long-term data remain limited.

IUD timing has its own exact term. Immediate postplacental insertion means within ten minutes after placental delivery, whether the birth is vaginal or cesarean. Immediate postpartum IUD insertion is category four when uterine infection, chorioamnionitis, postpartum endometritis, puerperal sepsis, or ongoing postpartum hemorrhage is present. The contraindication comes from the local uterine setting, not from the contraceptive hormone.

Now place age into the same framework.

Age alone does not contraindicate hormonal contraception.

Healthy nonsmoking women without specific cardiovascular risk factors can continue combined hormonal contraception until age fifty to fifty-five, category two.

The age number matters because fertility declines but persists.

Estimated sterility is seventeen percent at age forty, fifty-five percent by age forty-five, and ninety-two percent by age fifty.

Many patients remain capable of pregnancy while already experiencing perimenopausal bleeding or vasomotor symptoms.

Routine FSH testing to decide when hormonal contraceptive users are menopausal is misleading and is not recommended. Hormonal contraception can suppress or distort the interpretation of FSH, and a single number does not cleanly answer the clinical question.

Perimenopausal combined hormonal contraception can provide noncontraceptive benefit: better bleeding control, reduction in vasomotor symptoms, positive effects on bone mineral density, and reduced risk of endometrial and ovarian cancer.

The risk assessment changes when age travels with smoking, obesity, diabetes, hypertension, or migraine with aura.

In that setting, combined hormonal contraception may become category three or four.

Progestin-only pills, the implant, and levonorgestrel IUDs are options for older women with cardiovascular risk factors, category one to two.

The fifty-two milligram levonorgestrel IUD is particularly useful for perimenopausal bleeding.

DMPA in women older than forty-five without other cardiovascular risks is category two, but long-term use carries a bone-density concern. ACOG notes uncertainty about

whether bone mineral density returns to baseline before menopause after long-term DMPA use.

Now consider obesity.

Women of all weights can become pregnant. Obesity alone contraindicates no contraceptive method. Women with obesity can be offered all hormonal methods with reassurance that efficacy is not significantly affected by weight.

The evidence is nuanced.

A prospective cohort of one thousand five hundred twenty-three women using combined pills, the patch, or the ring found no significant difference in unintended pregnancy across BMI categories, with an overall three-year range of eight point four to eleven point zero percent.

Pharmacokinetic studies show that women with obesity may take twice as long to reach steady-state therapeutic steroid levels when starting pills or after the hormone-free interval.

Continuous pill use, or a thirty to thirty-five microgram ethinyl estradiol pill rather than a twenty microgram pill, may theoretically be more effective in selected women with obesity.

A larger cohort of more than fifty-two thousand women reported a slight increase in combined oral contraceptive failure among patients with BMI greater than thirty-five, hazard ratio one point five, with a ninety-five percent confidence interval of one point three to one point eight.

That difference disappeared when twenty-four-hour regimens were evaluated compared with twenty-one-hour regimens.

ACOG's final counseling message remains reassuring, with a research caveat for class three obesity, BMI forty or greater.

Combined hormonal contraception is category two for obesity. Obesity and combined hormonal contraception are independent VTE risk factors, but the absolute VTE risk with combined hormonal contraception in women with obesity is still less than VTE risk during pregnancy and puerperium in women with obesity. Older age, especially over thirty-five, should push the clinician to consider progestin-only and levonorgestrel IUD methods more strongly.

DMPA and the etonogestrel implant do not show higher pregnancy rates in overweight or obese women. The levonorgestrel IUD is protected by its local uterine mechanism; its efficacy does not rely on systemic drug levels and is not affected by BMI. Obesity also increases risk of abnormal uterine bleeding and endometrial hyperplasia, so progestin-containing methods, especially the levonorgestrel IUD, can add endometrial stabilization and protection.

Bariatric surgery teaches the difference between body size and absorption.

After malabsorptive procedures such as Roux-en-Y gastric bypass or biliopancreatic diversion, oral contraception is category three because absorption may be impaired. That includes combined oral contraception and progestin-only pills.

Nonoral methods are category one.

Restrictive procedures, including vertical banded gastroplasty, laparoscopic adjustable gastric band, and laparoscopic sleeve gastrectomy, do not create the same oral absorption concern, category one.

Depressed mood is brief but important because it corrects a common worry.

Depressive disorders are category one for all hormonal contraceptive methods. Depressive symptoms do not appear to worsen with any hormonal method, including DMPA. Combined hormonal contraception does not modify fluoxetine effectiveness, and SSRI or SNRI medications do not appear to interact with hormonal contraceptive metabolism.

St. John's wort deserves its own warning because it is an over-the-counter hepatic enzyme inducer. It can increase estrogen and progestin metabolism, increase breakthrough bleeding, and increase the likelihood of ovulation. ACOG rates concomitant St. John's wort use as category two for combined hormonal contraception, progestin-only pills, and the etonogestrel implant.

Migraine is an aura decision.

Migraine without aura is the most common migraine subtype, about seventy-five percent of cases. Menstrual migraine is a subtype of migraine without aura and may be treated with extended hormonal contraception to minimize endogenous hormonal fluctuations.

Aura is a complex of neurologic symptoms, usually visual, that usually occurs before the headache. It lasts five to sixty minutes. It may appear as zigzag lines spreading across the visual field, pins and needles, speech disturbance, or motor weakness.

For migraine without aura and no other stroke risk factors, combined hormonal contraception is category two. For migraine with aura, estrogen-containing contraception is category four because of ischemic stroke risk. Progestin-only methods are category one in migraine with or without aura.

The numbers make the rule durable.

Among combined hormonal contraceptive users, stroke risk rises with age from three point four events per one hundred thousand women-years in adolescents to sixty-four point four events per one hundred thousand women-years among women aged forty-five to forty-nine.

In a case-control study, the adjusted odds ratio for ischemic stroke was highest in women with migraine with aura using combined hormonal contraception, six point

one, with a ninety-five percent confidence interval of three point one to twelve point one.

Migraine with aura without combined hormonal contraception still carried increased risk, odds ratio two point seven.

Migraine without aura behaved differently: odds ratio one point eight with combined hormonal contraception and two point two without it, after adjustment for hypertension, diabetes, obesity, smoking, ischemic heart disease, and valvular disease.

Now hypertension.

The USMEC thresholds remain the contraceptive thresholds even after the two thousand seventeen A C C and A H A hypertension definitions lowered diagnostic categories. ACOG says little data exist for contraceptive risk in the one-thirty to one-thirty-nine systolic or eighty to eighty-nine diastolic range, so clinicians should continue to follow USMEC recommendations.

BP below one-forty over ninety allows any hormonal contraceptive method. Systolic one-forty to one-fifty-nine or diastolic ninety to ninety-nine makes combined hormonal contraception category three. Systolic one-sixty or greater, diastolic one-hundred or greater, or vascular disease makes combined hormonal contraception category four.

Hypertension and estrogen are an arterial-risk pairing. Among combined hormonal contraceptive users, the relative risk of acute myocardial infarction in women with hypertension is estimated to increase by a factor of twelve compared with women who are not using combined hormonal contraception.

Women on antihypertensive medications form a separate group.

Adequately controlled hypertension should reduce cardiovascular risk, but ACOG notes absence of data on combined hormonal contraception in this population.

Controlled hypertension on medication is therefore category three for combined hormonal contraception.

A narrow trial can be considered in women thirty-five or younger with well-controlled and monitored hypertension, otherwise healthy, no end-organ vascular disease, and no smoking.

If BP remains controlled after several months, continuation may be reasonable.

Low-dose combined oral contraceptives, thirty-five micrograms ethinyl estradiol or less, can still raise BP slightly, about eight millimeters mercury systolic and six millimeters mercury diastolic compared with copper IUD starters. BP should be checked at follow-up, and discontinuation considered if BP rises significantly without another cause.

Progestin-only pills do not appear to significantly affect BP or cardiovascular disease risk. BP measurement before or during use of progestin-only pills, DMPA, the implant, or levonorgestrel IUD is not necessary. DMPA is the exception to remember in severe hypertension: systolic one-sixty or greater or diastolic one-hundred or greater makes DMPA generally not advised, category three, because of theoretical unfavorable lipoprotein changes.

Diabetes follows the vascular-damage logic.

Hormonal contraception does not appear to affect carbohydrate metabolism in women without diabetes and is unlikely to precipitate diabetes. In uncomplicated insulin-dependent or non-insulin-dependent diabetes, no hormonal contraceptive method is contraindicated based on available data, category two.

The decision changes after more than twenty years of diabetes or with microvascular disease, meaning retinopathy, nephropathy, or neuropathy. Combined hormonal contraception becomes category three or four depending on severity. DMPA becomes category three because of concern for lipoprotein changes favorable to atherosclerosis. Progestin-only pills, levonorgestrel IUDs, and the implant are suitable alternatives.

Now cancer risk.

Family history of breast cancer and BRCA one or BRCA two mutation without personal breast cancer are category one for hormonal contraception. A family history, often defined as two or more close relatives with breast cancer, raises baseline breast cancer risk twofold to threefold, but ACOG cites a systematic review concluding that oral contraceptive use does not significantly affect breast cancer risk in women with family history.

In BRCA carriers, a two thousand thirteen meta-analysis found a nonsignificant association between oral contraceptive use and breast cancer, odds ratio one point two one, ninety-five percent confidence interval zero point nine three to one point five eight.

It also found a significant inverse association with ovarian cancer, odds ratio zero point five eight, ninety-five percent confidence interval zero point four six to zero point seven three, with stronger ovarian cancer protection suggested with longer combined hormonal contraceptive use.

Personal breast cancer changes the frame. Current or recent breast cancer makes hormonal contraception category four. No evidence of disease for five years or more makes hormonal contraception category three. The copper IUD is category one after treated breast cancer.

The fifty-two milligram levonorgestrel IUD can reduce tamoxifen-associated endometrial polyps, but its effect on breast cancer recurrence is uncertain.

One observational study compared seventy-nine premenopausal breast cancer patients using the fifty-two milligram levonorgestrel IUD with one hundred twenty who did not use it.

Overall recurrence was similar after a mean follow-up of two point eight years.

Among women who already had the IUD at breast cancer diagnosis and continued it, recurrence rose from sixteen point six percent to twenty-one point five percent, with marginal statistical significance.

The decision belongs case by case, often with oncology input, because the endometrial benefit and recurrence uncertainty both matter.

Gynecologic cancer brings a practical access point.

Cervical cancer screening is not required before contraception in asymptomatic patients.

If cervical, endometrial, or ovarian cancer is diagnosed and the patient is awaiting treatment, or if treatment will not create sterility, combined hormonal or progestin-only contraception, including continuation of a previously placed levonorgestrel IUD, is category one or two.

For endometrial hyperplasia or malignancy in patients who are not surgical candidates or who decline surgery, a progestin method, especially DMPA or the fifty-two milligram levonorgestrel IUD, may be used.

Gestational trophoblastic disease makes contraception important because a new pregnancy can confound hCG monitoring. After suction curettage, if hCG is falling or undetectable, or elevated without intrauterine disease evident or suspected, any hormonal method is category one or two. New IUD placement is category four when hCG is persistently elevated or malignant disease is present and intrauterine disease is evident or suspected, because of bleeding, infection, or perforation concerns.

Now the efficacy-failure zone: drugs and absorption.

Rifampin, rifabutin, enzyme-inducing antiepileptic drugs, and some antiretroviral medications can lower contraceptive steroid levels. In those settings, DMPA and levonorgestrel IUDs are category one. Combined hormonal contraception and progestin-only pills are generally category three because of failure risk. The etonogestrel implant is category two.

The enzyme-inducing antiepileptic drugs in this paper are carbamazepine, felbamate, oxcarbazepine, phenobarbital, phenytoin, primidone, and rufinamide. The named noninducers include clobazam, clonazepam, ethosuximide, ezogabine, gabapentin, lacosamide, lamotrigine, levetiracetam, pregabalin, tiagabine, topiramate, valproate, vigabatrin, and zonisamide.

Epilepsy counseling has two reasons for effective contraception. Seizure activity can worsen during pregnancy and affect maternal and fetal outcomes. Fetal antiepileptic

drug exposure carries a twofold to threefold increased risk of major congenital malformations compared with the general population, with higher rates reported with valproate or polytherapy.

If combined hormonal contraception is used despite enzyme-inducing antiepileptics, ACOG says efficacy may be improved with thirty to thirty-five micrograms ethinyl estradiol rather than lower doses, longer half-life progestins such as drospirenone, desogestrel, or levonorgestrel, and hormone-free intervals shorter than seven days. Those adjustments reduce vulnerability; they do not make combined contraception the preferred answer.

DMPA is strong in this setting because one hundred fifty milligrams intramuscularly every three months is substantially higher than needed to suppress ovulation. ACOG considers it the systemic hormonal method most likely to maintain efficacy with enzyme-inducing anticonvulsants, although direct pharmacodynamic proof is lacking. The fifty-two milligram levonorgestrel IUD remains highly effective with antiepileptic and other liver enzyme-inducing medications.

The implant sits between the vulnerable pills and the protected DMPA or IUD. Carbamazepine lowers etonogestrel levels, and implant pregnancies have been reported with enzyme-inducing antiepileptic medications. Because the implant is so effective at baseline, it remains category two rather than category three.

Lamotrigine is the directionality test. Estrogen-containing contraception reduces lamotrigine serum levels, and lamotrigine levels may rise during the hormone-free interval. Dose adjustment may be needed. Lamotrigine does not affect ethinyl estradiol levels, and its small effects on levonorgestrel levels are unlikely to affect contraceptive efficacy.

Rifampin and rifabutin are the important antibiotic-family exceptions because they lower estrogen and progestin levels.

Broad-spectrum antibiotics, antifungals, and antiparasitics generally do not interfere with oral contraceptive efficacy.

Pharmacokinetic studies have not shown decreased steroid levels with tetracycline, doxycycline, ampicillin, metronidazole, quinolones, or fluconazole.

Vaginal ring steroid levels were not reduced by ampicillin, doxycycline, or vaginal miconazole.

A short course of tetracycline did not affect patch pharmacokinetics.

Most antiretrovirals can be taken with all hormonal methods as category one or two; fosamprenavir is the exception named in the bulletin.

Anticoagulation completes the paper because it combines pregnancy danger, clot history, and bleeding symptoms.

All progestin-only methods are acceptable for women taking anticoagulants, category two. Warfarin is teratogenic, so reliable contraception has fetal-safety importance. Anticoagulated women may have heavy menstrual bleeding and, rarely, hemoperitoneum after ruptured ovarian cysts.

The fifty-two milligram levonorgestrel IUD and DMPA have been studied for heavy menstrual bleeding induced by anticoagulation therapy. DMPA also prevents ovarian cyst formation and rupture. Evidence has not revealed DMPA injection-site hematoma problems in anticoagulated women, although the evidence is limited.

Combined hormonal contraception can reduce menstrual blood loss but adds VTE risk. Because recurrent thrombosis risk is low while a woman is therapeutically anticoagulated, combined contraception can be considered case by case, category three, especially if alternatives are unacceptable or contraindicated.

Now seal the systems with cases.

Maya is twenty-seven and requests the combined pill because it helped a friend's menstrual migraine.

She describes bright zigzag lines that slowly spread across one side of her visual field for about twenty minutes before the headache.

That history is aura until proven otherwise.

Aura is a fully reversible neurologic symptom, usually visual, lasting five to sixty minutes.

Estrogen-containing contraception becomes category four because the mechanism is ischemic stroke risk.

Progestin-only methods remain category one.

Menstrual migraine without aura can fit extended hormonal contraception; migraine with aura removes estrogen.

Noa is eighteen days after a cesarean birth, breastfeeding, and wants to restart her combined ring.

She also had preeclampsia.

The ring is combined estrogen-progestin contraception.

Day eighteen is inside the first twenty-one postpartum days, where combined hormonal contraception is category four for everyone.

Breastfeeding and preeclampsia add caution, but the day already decides.

Progestin-only methods are available, and an IUD can be considered if there is no uterine infection, sepsis, or ongoing postpartum hemorrhage.

Hold the postpartum clock: twenty-one, thirty, forty-two, and twelve weeks.

Rina is thirty-one with SLE and a positive lupus anticoagulant. She asks for any progestin-only method because she has heard estrogen is the only problem. Positive antiphospholipid antibodies make combined hormonal contraception category four. Progestin-only methods and levonorgestrel IUDs are category three because her baseline thrombosis risk is high. If antibodies were negative and there were no cardiovascular risk factors, the categories would be far less restrictive. Lupus requires antibody-status thinking.

Dana is thirty-four, nonsmoking, and has repeated BP readings around one-forty-eight over ninety-four.

She wants the combined patch.

The threshold controls the answer.

Below one-forty over ninety, any hormonal method is allowed.

One-forty to one-fifty-nine systolic or ninety to ninety-nine diastolic makes combined hormonal contraception category three.

One-sixty systolic, one-hundred diastolic, or vascular disease makes it category four.

The patch follows combined hormonal contraindications.

Progestin-only or nonhormonal methods should be discussed while hypertension is addressed directly.

Liat uses combined oral contraception and receives doxycycline. Her friend tells her that antibiotics cancel the pill. The bulletin gives the resident a precise correction: broad-spectrum antibiotics generally do not interfere with oral contraceptive efficacy. Rifampin and rifabutin are the meaningful antimicrobial exceptions because they induce metabolism and lower steroid levels. Doxycycline belongs to the reassuring evidence, not the rifamycin exception.

Shira has epilepsy controlled with carbamazepine and wants a method she can forget.

Carbamazepine is an enzyme-inducing antiepileptic.

Combined hormonal contraception and progestin-only pills are category three because steroid levels can fall enough to risk failure.

DMPA and the levonorgestrel IUD are category one.

The implant is category two: carbamazepine lowers etonogestrel levels and pregnancies have been reported, yet baseline implant efficacy remains high.

The cleanest choices are DMPA or levonorgestrel IUD.

Tamar takes lamotrigine and starts a combined pill. Two months later, she reports breakthrough neurologic symptoms. This is the reverse interaction. Estrogen-containing contraception reduces lamotrigine serum levels, and lamotrigine levels may rise during the hormone-free interval. Neurology coordination and dose

adjustment may be needed. Lamotrigine itself does not meaningfully lower ethinyl estradiol, and its small effects on levonorgestrel are unlikely to reduce contraceptive efficacy.

Yael has BMI forty-two and wants contraception.

Her cousin had Roux-en-Y gastric bypass and was told to avoid pills, so Yael assumes BMI alone makes pills ineffective.

Obesity alone contraindicates no method, and combined hormonal contraception is category two.

Evidence generally reassures that hormonal efficacy is not significantly affected by weight, with caveats for class three obesity and pill pharmacokinetics.

Roux-en-Y is a different mechanism: malabsorption.

After malabsorptive bariatric surgery, oral combined and oral progestin-only contraception are category three, while nonoral methods are category one.

Moran has a BRCA one mutation and no personal breast cancer.

Her sister developed breast cancer at thirty-eight.

Family history and BRCA mutation without personal breast cancer are category one for hormonal contraception.

The cited meta-analysis in BRCA carriers found a nonsignificant breast cancer association and a significant ovarian cancer risk reduction with oral contraceptive use.

Current or recent breast cancer would make hormonal contraception category four.

Five years disease-free would be category three.

Copper IUD remains category one after treated breast cancer.

Hadar takes warfarin after VTE and has heavy menstrual bleeding.

Warfarin is teratogenic, so reliable contraception matters.

Anticoagulation also worsens menstrual bleeding and can make ovarian cyst rupture dangerous.

The fifty-two milligram levonorgestrel IUD provides reliable contraception and reduces menstrual blood loss.

DMPA can reduce bleeding and prevent ovarian cyst formation and rupture.

Combined hormonal contraception can reduce bleeding but adds VTE risk; while therapeutic anticoagulation lowers recurrence risk, combined contraception remains category three and case-by-case.

Orna is forty-seven, healthy, nonsmoking, and wants to continue combined hormonal contraception because it controls bleeding and vasomotor symptoms.

Age alone does not contraindicate hormonal contraception.

Healthy nonsmoking women without cardiovascular risk factors can continue combined hormonal contraception until age fifty to fifty-five.

FSH testing in hormonal contraceptive users is misleading and is not recommended.

Smoking, hypertension, diabetes, migraine with aura, obesity with additional risk factors, or vascular disease would change the risk calculation.

Michal is recovering after suction curettage for gestational trophoblastic disease, and hCG is being monitored.

Contraception protects the interpretation of hCG by preventing a new pregnancy from confusing surveillance.

If hCG is falling or undetectable, or elevated without intrauterine disease evident or suspected, hormonal contraception is category one or two.

New IUD placement is category four when hCG is persistently elevated or malignant disease is present and intrauterine disease is evident or suspected.

The final compression is this.

When the patient has clot, arterial, aura, early postpartum, severe blood pressure, or vascular disease physiology, combined hormonal contraception is the first method family to scrutinize because estrogen is the dangerous component.

When the patient takes enzyme-inducing drugs or has malabsorptive surgery, the clinical problem is failure. DMPA and levonorgestrel IUDs are protected against enzyme induction; oral methods are vulnerable after malabsorptive bariatric procedures.

When the patient has breast cancer risk, separate family history or BRCA mutation from personal breast cancer. Family history and BRCA without personal disease are category one. Current or recent breast cancer is category four. Five years disease-free is category three. Copper IUD is category one after treated breast cancer.

When the patient is postpartum, use the clock. Twenty-one days marks the end of the absolute combined hormonal prohibition for some patients. Thirty days matters for breastfeeding. Forty-two days ends the higher postpartum combined-hormonal restriction for many patients. Twelve weeks is the approximate return-to-baseline VTE horizon.

When the patient has hypertension or diabetes, name the severity boundary. One-forty over ninety and one-sixty over one-hundred are contraceptive eligibility thresholds. More than twenty years of diabetes or microvascular disease changes the method discussion from uncomplicated metabolic disease to vascular disease.

The strongest clinical habit from ACOG Practice Bulletin two hundred six is mechanism before method. Once the mechanism is clear, the category becomes far easier to retrieve, and the counseling becomes safer, shorter, and more honest.